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Dr. Matthew Balcarras  
Editor, Interactive Journal of Medical Research  
JMIR Publications

RE: Revised Manuscript (R1) Resubmission: ms#96541, Decision D (major revision and re-review)

Dear Dr. Balcarras,

I am writing to resubmit the manuscript “Healthcare Analytics Challenges: A Three-Pillar Framework Connecting Analytics Maturity, Workforce Agility, and Technical Enablement” (ms#96541) as a *Viewpoint* article, revised in response to Decision D (major revision and re-review).

## Response to Decision D

I am grateful to you and to the two external reviewers for the constructive and detailed assessment. The reviews materially strengthened the manuscript, and I have addressed every editorial and reviewer comment. A full point-by-point response accompanies this resubmission; the summary below highlights the principal changes.

## Summary of Revisions

*Engagement with recent literature and evidence (Editor; Reviewer Q3):*

- Added empirical evidence that human oversight measurably improves AI-generated analytics, including a prospective study in which human-verified large-language-model output exceeded a human-only workflow, and a text-to-SQL validation gate that improved semantic correctness.
- Added database query-log evidence on query reuse and diversity to address the scope and practicality of the framework.
- In total, 14 references spanning 1994-2026 were added, the majority from 2016-2026.

*Conceptual clarity (Reviewer T1):*

- Added explicit, cited definitions of the three title constructs (analytics maturity, workforce agility, and technical enablement) at the head of each evidence pillar, grounded in the established literature.

*Methodological caveats (Reviewer Q1, Q4):*

- Added a self-selection-bias caveat to the analytics-maturity evidence, grounded in the organizational-survey methodology literature.
- Added a Limitations paragraph acknowledging that the framework complements, rather than replaces, investment in workforce training and analytics literacy.

*Currency and consolidation (Reviewer Q6; Reviewer T2, T4):*

- Replaced aging workforce statistics with contemporary (2021-2025) evidence; the single remaining two-decade-old reference is now framed explicitly as a historical benchmark.

- Consolidated duplicated turnover statistics to a single mention.

*Figure revision (Reviewer T3):*

- Revised Figure 1 to add a feedback edge from Organizational Memory to the Knowledge Base, depicting continuous learning rather than a static knowledge base.

*Formatting and compliance (Editor):*

- Converted all in-text citations to numbered square brackets.
- Removed heading numbering and the italic sub-table titles that stood in place of subheadings.
- Added a Funding statement distinct from the Acknowledgments, and a generative-AI disclosure within the manuscript.
- Completed all reference details (volume, issue, pages, DOI) and the Abbreviations list, with acronyms defined at first use.

The revised manuscript remains a *Viewpoint* of 4,532 body words (within the 5,000-word limit) with an unstructured abstract of 304 words.

## Article Type

This submission is a *Viewpoint* article: an evidence-based scholarly perspective on mitigating institutional amnesia in healthcare analytics, grounded in peer-reviewed literature and knowledge management theory. As the Viewpoint article type does not permit IMRD section headings, descriptive thematic headers are used throughout, as detailed in the point-by-point response.

Thank you for your continued consideration.

Respectfully,

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